5.3 Traumatic Brain Injury

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say he fell and hit his head **Objective** • GCS 11 • Pupils unequal • Vomiting • Headache	**ACTUAL** **Decreased intracranial adaptive capacity** related to brain injury as evidenced by reduced GCS, pupil changes and vomiting.	GCS stays stable or gets better, with no signs of raised ICP.	• Check GCS, pupils and vital signs as ordered (often every hour). • Head of the bed up to 30 degrees, head kept in the midline. • Do not let the neck flex, and keep tube ties loose. • Keep him well oxygenated with a normal CO_2. • Report a falling GCS, new pupil changes, or a rising BP with a slowing HR.	GCS stable or better: **goal met**.
Subjective • "My head is pounding." **Objective** • Grimacing • Pain 7/10	**ACTUAL** **Acute pain** related to head trauma as evidenced by headache and grimacing.	Headache 3/10 or less within 1 hour of analgesia.	• Give the prescribed analgesia, choosing one that will not hide neurological signs. • Dim the lights and cut the noise. • Keep the head of the bed at 30 degrees. • Recheck pain each time you do neuro obs.	Pain controlled: **goal met**.
Subjective • *No specific complaint* **Objective** • Head injury • Restless	**RISK** **Risk for injury** as evidenced by risk factors: altered level of consciousness and possible seizures.	No injury.	• Padded side rails, as policy allows. • Bed kept low. • Watch for seizures.	No injury: **goal met**.